Sample 7-minute ISBAR: Jayden

Jayden Mitchell · 19-year-old man · Inpatient spinal injuries rehabilitation unit, public hospital · about 1046 spoken words (≈ 7.5 min at 140 words per minute)

I Introduction

55 words

"Hi, I'm the occupational therapy student who has been running Jayden Mitchell's computer access and splinting session in the therapy room. Jayden is 19 and has C6 AIS A tetraplegia from a diving injury 10 weeks ago. I need help now, because I think Jayden may be developing autonomic dysreflexia, which is a medical emergency."

S Situation

142 words

"About twenty minutes into the session Jayden suddenly developed a pounding headache, which he described as 'like my head's going to explode'. His face and neck are flushed and he is sweating on his forehead even though the room is cool. His nose has suddenly blocked, his vision has gone blurry, and he has goosebumps on his lower legs.

"I've also found the catheter tubing to his leg bag folded tightly under the edge of the wheelchair frame, and the leg bag is almost empty.

"Jayden is putting it down to staying up late gaming and says it will pass. His mum has just arrived wanting to talk about home modifications and his mate is on his way up. There is no nurse in the therapy room, so I have stopped the session and I need nursing and medical help straight away."

B Background

170 words

"Jayden had surgical stabilisation in the acute hospital and moved to the spinal unit four weeks ago. He has a suprapubic catheter draining to a leg bag, wears an abdominal binder when sitting, and has his bowel programme on alternate evenings. His chart lists his usual blood pressure as around 100/65.

"He is described as medically stable; physiotherapy has documented that he tolerates sitting for most of the day, and this morning's nursing note says 'comfortable, no concerns'. Before his injury he lived with his mum, Leanne, in a highset house in Ipswich and was a second-year electrical apprentice.

"There are some important gaps. There are no documented observations since 8 am, so I don't know his current blood pressure. I don't know when his leg bag was last emptied, whether his bowels are due, or whether he has had episodes like this before, although he said, 'It always does,' which suggests he may have. I also need to confirm the unit's autonomic dysreflexia procedure, which I haven't used before."

A Assessment

261 words

"My main concern is that Jayden's signs fit together. A pounding headache, flushing and sweating above his level of injury, a blocked nose, blurred vision and goosebumps below his level are the classic warning signs of autonomic dysreflexia. It occurs with injuries at T6 or above, usually triggered by something below the level of injury that he can't feel. The most

common trigger is a full bladder, and the kinked catheter tubing with an almost-empty leg bag strongly suggests his bladder hasn't been draining.

"This matters because his blood pressure can climb quickly to dangerous levels. Guidelines usually treat a rise of 20 to 40 mmHg above baseline as significant. Jayden's baseline is only around 100/65, so a reading that looks 'normal' for most people could already be high for him. Untreated, it can lead to seizures, stroke or other serious complications. So this is an immediate safety priority, ahead of anything else in the session.

"Jayden is relaxed and explaining it away, and the notes say he is medically stable, but those findings are from earlier today and don't tell us what is happening now. Saying 'It'll pass, it always does' also worries me, because recurring episodes that aren't reported need to be followed up.

"As a student, I can't diagnose this or decide medical management, and I don't have a blood pressure reading, but the signs are clear enough to start first-aid steps and escalate rather than wait.

"Leanne's NDIS and ramp questions, Tyler's visit and the splint are all important, but they can wait until Jayden is stable."

R Recommendation

418 words

"First, I'd stay with Jayden and call for nursing help immediately using the call bell, or the emergency call if that is the unit's procedure. A nurse can check his blood pressure straight away and repeat it every few minutes.

"While help comes, I'd keep him sitting upright and not lie him down, because sitting up helps lower blood pressure. I'd lower his legs if they are raised, loosen his abdominal binder and anything tight such as the leg bag straps, and free and straighten the kinked tubing so his bladder can drain, following the unit's procedure. I'd ask Leanne to wait outside for a few minutes and let Tyler know to hold off.

"Second, I'd like your help to escalate this to the nurse in charge and the medical team, because Jayden needs medical review now. Nursing can follow the unit's autonomic dysreflexia protocol, check whether the bladder is draining and look for other triggers such as bowel or skin. Medical staff can decide on any medication if his blood pressure stays high and investigate further if needed. I'd ask for his observations to be continued after it settles, because the signs can come back.

"Third, once he is stable I'd document clearly what I saw, what I did and when, and complete an incident report if required.

"For OT, my principal short-term assessment would be observed performance of his wheelchair set-up and daily routine, checking where his catheter tubing and leg bag sit, how the binder fits and whether his positioning or the desk set-up for computer use could cause kinks. Because Jayden will rely on carers for much of his physical care, I'd work with nursing on education so he can recognise the signs and direct others exactly what to do.

"Longer term, I'd use the COPM, because Jayden has told us gaming and staying connected with his mates matter most to him. That would help prioritise computer access, writing, study or work and home planning around his own goals.

> "Later, and with Jayden's consent as an adult, I'd meet with him and Leanne about the highset home, ramps or a lift, and NDIS access, ideally with the social worker, since this was a diving injury and wouldn't fall under NIISQ.
>
> "I'd use the NSW Agency for Clinical Innovation guidance on treating autonomic dysreflexia to guide the first-aid steps and the education. For now, though, my priority is to keep Jayden upright, relieve the blocked drainage and get nursing and medical help to him immediately."

Coaching notes *not spoken*

The immediate priority

Jayden has the classic signs of autonomic dysreflexia with an obvious trigger, the kinked catheter tubing. This needs immediate first-aid action and urgent nursing and medical escalation before anything else.

Distractors in this case

- **"I stayed up till three gaming"** — tiredness doesn't explain flushing above the injury level, a blocked nose and goosebumps below it.
- **"Medically stable", "comfortable, no concerns", physio sitting tolerance** — these are earlier findings and say nothing about what is happening now.
- **Leanne's NDIS and ramp questions** — important later, with Jayden's consent, but not now.
- **Tyler's visit and finishing the splint** — the session pauses; the goals can wait an hour.
- **"It'll pass, it always does"** — this is a warning sign of possible unreported episodes, not reassurance.

Missing information to clarify

- Current blood pressure compared with his baseline of around 100/65
- When the leg bag was last emptied; bowel status
- Any previous episodes and how they were managed
- The unit's autonomic dysreflexia protocol and any prescribed medication
- Jayden's wishes about involving Leanne in planning

Useful references

- NSW Agency for Clinical Innovation, *Treatment of autonomic dysreflexia for adults and adolescents with spinal cord injuries*
- Consortium for Spinal Cord Medicine, *Evaluation and Management of Autonomic Dysreflexia and Other Autonomic Dysfunctions* (clinical practice guideline)

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